PATIENT RIGHTS ADVOCATE.ORG

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June 16, 2025

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Submitted electronically via: http://www.regulations.gov

Re: CMS-0042-NC - **Request for Information: Health Technology Ecosystem [RIN 0938–AV68]**

Dear Drs. Oz and Keane:

Thank you for the opportunity to submit comments in response to the Request for Information: Health Technology Ecosystem (the "RFI"). We particularly appreciate the focus on the impact of technology on patients. We agree that the Department of Health and Human Services ("HHS") has an opportunity to ensure that adoption of technology can empower patients to make better decisions for their health and well-being, and must include actual, upfront prices – not estimates – for all consumers.

PatientRightsAdvocate.org ("PRA") is a 501(c)(3) nonprofit, non-partisan organization that provides a voice for healthcare consumers – patients, employees, employers, and taxpayers – to have access to patient data, price, quality, and outcomes transparency, such that patients can make the best informed decisions as they take care of their physical, mental, and financial health. Empowered, informed consumers will choose to lower their costs by comparing and shopping in a functional, competitive marketplace for the best quality of care at the lowest possible price. Upfront prices will also allow for billing and payment integrity, enabling employer and union group health plans to pre- and post-audit their claims data, and improve their benefit plan design.

General Comments

Patients need the ability to own their longitudinal records with full visibility into all health records, including labs, radiology, images, prescription fills, and care plans. We envision a future where a patient's data flows at their direction and choice – whether they choose to store their record within a provider's patient portal, in their mobile app, third party application, or by some innovative mechanism yet to be invented. Data should be automatically updated, free to the patient, as it is digital. It should be provided in a machine-readable and analyzable format so that patients, caregivers, and providers can see trends and best manage their care as they receive it. Additionally, it should be bi-directional and global, where patients can provide their comprehensive records, results of self-monitoring, care plans, and other information from either conventional or alternative providers.

With regard to price information, PRA urges HHS to fully implement the Presidents' Executive Order to Making America Healthy Again by Empowering Patients with Clear, Accurate, and Actionable Healthcare Pricing Information (the "Price Transparency EO") and implement ***radical*** price transparency by strengthening existing regulations and enforcement and to ensure that calls for technology and standards are not used to delay implementation of these rules. Patients must have actual, upfront prices in advance of care. For too long, hospitals and health plans have failed to provide accurate and complete price information for consumers, despite regulatory requirements to do so. Health care organizations hide behind complex payment arrangements that they create and assertions that more technology and standards are needed. It is time for HHS to demand that health care organizations provide actual, binding prices, in dollars and cents, to patients who need this information to make critical healthcare decisions. The health care sector seems to think we have sufficient standards to support payment by health plans to health care providers and hospitals. These same standards can and should support needed disclosure of information to patients and purchasers.

Furthermore, we urge HHS to immediately promulgate regulations to comply with the requirement in the No Surprises Act (NSA) for the provision of Advanced Explanations of Benefits (AEOBs) for all individuals. To ensure that insured individuals are afforded protections against surprise health care costs, the NSA includes provisions that require providers and facilities to furnish "good faith estimates" or "GFEs" to uninsured (or self-pay) individuals upon their request at the time of scheduling the item or service and to the group health plan or issuer in the case of insured individuals. These provisions were initially intended to apply with respect to plan years (in the individual market, policy years) beginning on or after January 1, 2022. In light of the significant delay and long-past Congressional deadline, the importance of AEOBs to advancement in patient price transparency as a critical part of the NSA's patient protections, and the Price Transparency EO, now is the time for these requirements be adopted so that patients have the information they need to make health care decisions.

Plans and providers already have processes in place for the widespread practice of providing explanations of benefits (EOBs). Group health plans and health insurance issuers have processes and technology in place to receive information about items and services from health care providers and health care facilities and to provide EOBs based on this information. CMS must require group health plans and health insurance issuers to provide similar information to individuals in advance of care. The health care industry has been alleging that new standards

need to be in place to support AEOB and have been using this as an excuse to encourage CMS to delay the Congressional mandate to provide AEOB's to patients. Healthcare providers and facilities and health plans have been engaging in electronic communications for financial and administrative transactions for over 20 years. CMS regulations should build on and leverage these existing processes, communications, and standards to provide information and protections for individuals, without delay.

Advancements in technology must enable this critical price information to be available to individuals easily and timely, not be used as a delay tactic.

Detailed Comments

1. Patient Needs

> *PC-1. What health management or care navigation apps would help you understand and manage your (or your loved ones) health needs, as well as the actions you should take?*
> *a. What are the top things you would like to be able to do for your or your loved ones' health that can be enabled by digital health products?*
> *b. If you had a personal assistant to support your health needs, what are the top things you would ask them to help with? In your response, please consider tasks that could be supported or facilitated by software solutions in the future.*

Patients need accurate real-time price information to make informed decisions about their care and to ensure that they are not overcharged for needed services. Prices give consumers the ability to comparison shop and design health plans, choosing better quality and lower cost providers. Importantly, prices can be compared to claims data to prevent and remedy rampant overcharges, spread pricing, and to expose errors and fraud that occur in the shadows of the current opaque system. In order to have integrity and trust and a true competitive healthcare market, both patients and employer and union health plans need to be fully informed of accurate and actual price quotes, bills to match that price, and reconciliation of employer claims and funds transfer of payments, just as in any other functioning market.

We encourage HHS to hold providers and payers accountable for providing this information in compliance with legislative and regulatory requirements and enable this information to be presented to patients in a way that enables them to compare the AEOB with the EOB and the bill that they receive after they receive health care services. This data must also include dates and provenance that enable patients to hold healthcare providers and plans accountable for the prices provided to the patient in advance of care.

> *PC-2. Do you have easy access to your own and all your loved ones' health information in one location (for example, in a single patient portal or another software system)?*
> *a. If so, what are some examples of benefits it has provided?*
> *b. If not, in what contexts or for what workflows would it be most valuable to use one portal or system to access all such health information?*
> *c. Were there particular data types, such as x-rays or specific test results, that were unavailable? What are the obstacles to accessing your own or your loved ones' complete*

health information electronically and using it for managing health conditions or finding the best care (for example, limitations in functionality, user friendliness, or access to basic technology infrastructure)?

Not knowing prices and not being able to shop for health care puts a huge strain on working American families.

For instance, Shameeza Singh, born and raised in Queens, NY, is a married working mother of three children, ages 11, 10 and 7. All three children have a rare blood disease. Her oldest son also fought leukemia (blood cancer). She and her husband have multiple health issues, and he recently underwent three corneal transplants. They have thousands of dollars of medical debt. No one can predict their future health issues, but she would certainly benefit from being able to shop and compare prices to avoid additional medical debt.

Similarly, when Paige and Adam Nielson, of Denver, Colorado, had their first baby last year, they were thrilled to be starting what they hoped would be a growing family. But after they totaled all the bills, which kept coming and coming, associated with having a healthy full-term baby, they decided to put that plan on pause. The cost for a vaginal delivery totaled over $70,000. Though insurance helped, they still paid nearly $30,000 out of pocket. If they could have known the prices for all the different fees they were ultimately charged, they would have shopped or negotiated for more reasonable prices. Both working professionals -- she's a veterinarian and he's a geologist -- with insurance, they have decided to hold off having more children until they can know with some financial certainty what it will cost.

PC-4. What features are missing from apps you use or that you are aware of today?
a. What apps should exist but do not yet? Why do you believe they do not exist yet?
b. What set of workflows do you believe CMS is uniquely positioned to offer?

Patients need apps that easily enable them to access actual prices, information about any medical management or prior authorization requirements, AEOBs, good faith estimates, and bills. This information should all be readily available to patients in a simple way that enables comparison fo GFEs/AEOBs and bills. While patients are able to access their medical records through apps and sites like MyChart, these apps lack the ability to provide patients with an upfront, binding price; the most they provide is an estimate, with no accountability. CMS has all the authority it needs to publish strong regulations on price transparency and AEOB and to enforce these rules so that technology developers can build tools to support patients that can be integrated with the already existing patient-facing applications.

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2. Data Access and Integration

PC-8. In your experience, what health data is readily available and valuable to patients or their caregivers or both?

a. What data is valuable, but hard for patients and caregivers, or app developers and other technical vendors, to access for appropriate and valuable use (for example, claims data, clinical data, encounter notes, operative reports, appointment schedules, prices)?
b. What are specific sources, other than claims and clinical data, that would be of highest value, and why?
c. What specific opportunities and challenges exist to improve accessibility, interoperability and integration of clinical data from different sources to enable more meaningful clinical research and generation of actionable evidence?

Patients need ready access to actual prices, not estimates. With regard to hospital price transparency rules, we encourage CMS to:

- Clarify that all standard charges are required to be posted for all items and services, unless an item or service is not furnished by the hospital, and propose removing the term "as applicable."
- Require the machine-readable files to follow a standard schema that is both human-readable and machine-readable formats.
- Require attestation by senior hospital executives and deem them material to payments from the federal government to incorporate liability under the False Claims Act.
- Strongly enforce the Hospital Price Transparency Rule.

In addition, we urge CMS to act swiftly to adopt final rules requiring AEOBs. AEOBs can play a crucial role in providing clear, accurate, and usable information about the price of care for insured individuals in advance of furnishing of health care, to enable individuals to make informed decisions and protect them against surprise bills that are common today. Health care providers are already required to provide GFEs to uninsured/self-pay individuals. CMS should require that AEOBs and GFEs must be provided upon request and upon an insured individual's scheduling of an item or service to ensure timely access to this important information.

Historically, very little meaningful health data has been readily available to patients, despite this same data flowing freely for billing purposes. America's healthcare system has long operated under a "public model" where individual consumer preferences and competitive pricing were not primary drivers. Providers and health plans have considered pricing information proprietary and actively blocked access to it. Patients need ready access to actual prices, not estimates. Patients and caregivers who are making healthcare decisions impacting not only their health but their financial futures should have access to the same data that enables our $4.5 trillion healthcare system to process billions of claims.

The Trump Administration's price transparency requirements represent a fundamental shift from treating healthcare prices as trade secrets to recognizing them as essential consumer information. But despite over $35 billion in federal incentives for health information technology, meaningful price data remains locked away. Healthcare entities actively block patient access to pricing information as a business strategy, protecting market position over patient needs. This obstruction is so systematic that it required congressional intervention through the 21st Century Cures Act to impose penalties for information blocking.

The Transparency in Coverage rules already mandate disclosure of negotiated rates, yet compliance remains woefully inadequate. Health plans possess exact pricing for every provider and service; they use this data billions of times annually for claims processing. The same machine-readable files that could enable price shopping and competition are instead delivered in unusable formats, with missing data, or buried in ways that defeat transparency. This isn't a technical challenge; it's willful obfuscation.

Currently available data consists mainly of vague estimates and incomplete information that excludes facility fees and ancillary charges. The valuable but restricted data includes actual negotiated rates, complete episode pricing with all fees included, binding price quotes (which CMS should require to be binding for at least 30 days), and cash price alternatives that should always be available to promote competition. Beyond claims and clinical data, the most valuable sources include machine-readable files with all standard charges and negotiated rates as required by Transparency in Coverage rules, all-inclusive bundled pricing, third-party accessible data formats that enable innovation, and complete pricing information regardless of insurance status.

Key opportunities include transforming healthcare through price competition, enabling informed patient-provider conversations about value, spurring quality improvement through consumer demand, and facilitating third-party innovation. Primary challenges are deliberate information blocking, false complexity claims to avoid compliance, incomplete or unusable data formats, and resistance from a system built on price opacity.

CMS should clarify that all standard charges must be posted for all items and services, require standardized machine-readable file formats, mandate inclusion of all facility fees and ancillary charges, move toward binding price quotes valid for 30+ days, ensure cash prices are always available, require executive attestation with False Claims Act liability, and strongly enforce transparency rules with meaningful penalties.

The Transparency in Coverage rules already require disclosure of this pricing data. The challenge is not creating new data sources but ending the practice of treating public pricing information as proprietary, hidden trade secrets. Every American deserves the same pricing clarity when shopping for healthcare that they receive when shopping for any other service. This fundamental change can finally move American healthcare toward a system where competition on price drives competition on quality, innovation, and value.

> *PC-10. How is the Trusted Exchange Framework and Common Agreement™ (TEFCA™) currently helping to advance patient access to health information in the real world?*
> *a. Please provide specific examples.*
> *b. What changes would you suggest?*
> *c. What use cases could have a significant impact if implemented through TEFCA?*
> *d. What standards are you aware of that are currently working well to advance access and existing exchange purposes?*
> *e. What standards are you aware of that are not currently in wide use, but could improve data access and integration?*
> *f. Are there redundant standards, protocols, or channels that should be consolidated?*

g. Are there adequate alternatives outside of TEFCA for achieving widespread patient access to their health information?

We recommend that ONC keep the patients and their families front of mind with the goal to deliver the best quality of healthcare at the lowest possible price. We agree with the goals and vision of the TEFCA – to enable a health system where individuals are at the center of their care, and where patients, caregivers, and providers have the ability to securely access and use health information from different sources. We look to ONC to set the guardrails and provisions that support patient access, yet not be overly prescriptive in the mechanism defining standards and technology that limit innovation.

We support the TEFCA's emphasis of the right of individual access under HIPAA and the extension of access to the patient's personal representative, without special effort, in alignment with the goals of the 21st Century Cures Act. Providing open API's and allowing for future novel approaches are key to building an innovative, competitive marketplace in health technology network data sharing and applications. This should include clinical data as well as claims, benefit, and cost data.

In addition to automatic, digital delivery to the patient, we are supportive of the provisions that QHINs must respond to a query/pull request for individual access, free of charge, regardless of whether that query was initiated directly by the patient or a third-party application selected by the patient.

We urge ONC to offer the stakeholder community additional periods for public comment, both after the issuance of information blocking regulations later this year, and after publication of the next iteration of the Common Agreement. We also urge ONC to engage directly with patients, caregivers, and innovative technology developers to follow this process and provide feedback in order to understand their needs.

While we are supportive of the goals of TEFCA , we believe the TEFCA must have rules that are flexible enough to support interoperability that has been organically developing and innovation that will develop in the future. We are concerned about the overly prescriptive, top-down approach of the TEFCA, and its impact on innovation in the market. The requirements of the TEFCA will be burdensome for many existing networks to adopt and may lead to provider drop-out – an unintended consequence that would not achieve ONC's vision. It is also unclear how app developers that support innovative uses of health data by patients and caregivers connect with this network, whether they must participate in a QHIN, and how to ensure that the "on ramp" equally works for patients that wish to access their data.

This TEFCA model appears to potentially favor and further add to already advantaged entrenched oligopolies. It continues to put the control of patient data primarily into the hands of a few select, consolidating, sectored companies and organizations. The creation of the RCE gives even further concentration and substantial control over the future of our citizens' health information. We encourage ONC to not overregulate, to not favor the few, and to not potentially smother innovation. We also encourage ONC to develop an approach that does not vest significant power in one organization, an RCE, and a limited number of QHINs that will risk

cementing current approaches and not make way for innovation. All of the intermediaries, including TEFCA QHINs and Participants, create obstacles for patients to obtain information and take advantage of patients' information for their own financial benefit. TEFCA policies are designed by large, dominant companies that create rules that benefit them and not patients.

Finally, CMS should focus on access to critical health information that is important for patients and purchasers and make this information directly available to them. Arrangements that put a few large technology companies in the middle make it harder for patients to get the information they need. If CMS finalizes and enforces strong rules for price transparency rather than promoting large intermediaries, patients will be able to have direct access to this critical information.

> *PC-12. What are the most valuable operational health data use cases for patients and caregivers that, if addressed, would create more efficient care navigation or eliminate barriers to competition among providers or both?*
> *a. Examples may include the following:*
> *(1) Binding cost estimates for pre-defined periods.*
> *(2) Viewing provider schedule availability.*
> *(3) Using third-party apps for appointment management.*
> *(4) Accessing patient-facing quality metrics.*
> *(5) Finding the right provider for specific healthcare needs.*
> *b. What use cases are possible today?*
> *c. What should be possible in the near future?*
> *d. What would be very valuable but may be very hard to achieve?*

A competitive market for health care requires price transparency to enable patients and purchasers to shop for care. Arming healthcare consumers with real, binding prices will enable them to shop for the best quality of care at the lowest possible price and substantially reduce their costs of care. Binding prices will give consumers financial peace of mind that their final bill will match the quoted price and immediate recourse if they do not match. Upfront prices will hold hospitals and health insurers accountable for widespread price gouging, up-charging, and billing fraud that financially devastates millions of Americans.

Individuals have the right to access and control their healthcare data including adjudicated claims data, such as provider remittances and beneficiary or enrollee cost-sharing data; encounters from capitated providers; and clinical data, including MRIs, films, images, laboratory results, care plans, physician notes, etc., easily available to them in a usable electronic, machine- and human-readable form. In many contracts with PBMs and TPAs, employer and union group health plans are unknowingly granting middlemen access to their own plan members' data, yet those same members encounter difficulties trying to access their own health information. We encourage HHS to eliminate the perverse incentive of the patient becoming the product as his or her data is sold and remarketed without their knowledge.

Conclusion

We applaud HHS for its interest in the impact of technology on consumers and for its efforts to promote transparency and to make price information available for patients in free, machine-readable formats. Strong regulations and enforcement will ensure that health care consumers have reliable information they need to make decisions about coverage, treatment, and out-of-pocket costs. Patient access to price and payment information is critical to improving health care clinical outcomes and cost reductions, and strong rules will enable technology developers to make this information available to consumers in ways that support consumer decision making. Public disclosure of price information will increase competition within the health care market and will allow patients to shop for services and items based on price and quality.

Thank you for the opportunity to submit comments on this important RFI.

Sincerely,

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